جوستاف روسي انترناشيونال - مصر (EPE) * **طلب علاج أول مرة / متردد**

إسم المريض رباعياً : رشا محمد كامل محمد ابراهيم
المحافظة : اسوان **الحي / المدينة :** ادفو **العنوان :** -
رقم البطاقة : 28401200104102 **رقم التليفون :** 01124480867 **تاريخ تسجيل الإستمارة :** 17-10-2026
العمر(تاريخ الميلاد) : 20-01-1984 **المهنة :** استرجي

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)
نتائج الأبحاث : (مرفق)
التشخيص النهائي : سرطان بالثدي (مبادرة)
خطة العلاج : علاج مبادرة سرطان الثدي علاج تدعيمي وتلطيفي [6579] **قسم العلاج :** الاورام
التعليق على التشخيص : ----------

خطاب إداري

هذا الرقم القومي يتبع منظومة التأمين الصحي الشامل جهة الاختصاص

جوستاف روسي - مصر

Name: رشا محمد كامل محمد ابراهيم
National ID: 28401200104102



تقرير طبي

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026

وبعد الكشف والفحوصات والمتابعة وجد انة

المريضة تعاني من ورم غير حميد بالثدى و تحتاج الي Supportive and palliative Treatment

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين





National ID · Front & Back
[[DOCTYPE:ID_FRONT_BACK]]

جوستافروسي

مبادرة رئيس الجمهورية لدعم صحة المرأة المصرية

الست المصرية همه صحة مصر

MDT FORM

31/12/20 G.

Name		MRN	
Age		ID	
Menopausal status		Phone	
NRS SCORE		Marital status	
BMI		Family history	
CO morbidity		REASON OF MDT	

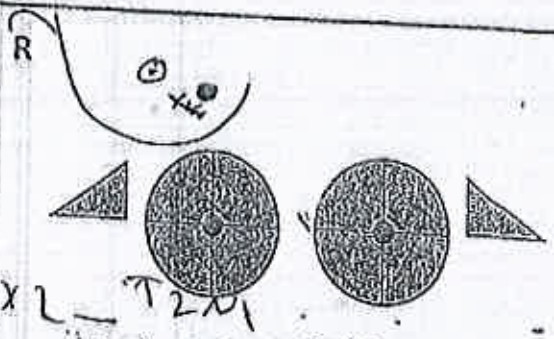
Clinical Data

BX:

ER:	PR:	HER 2:	Ki67:
-ve	-ve	-ve	26%

Sonomammography:

Mets Workup:



Recommendations:

MDT OUTCOME & PLAN

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
3/12/2020					

Name: رشا محمد كامل محمد	
MR. : 5067	

Detailed Medical Report

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من		**بشأن تقرير طبي للمريض**	
بقسم:		**رقم طبي:**	

ننشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول : **تاريخ وساعة الخروج :**

وبعد الكشف والفحوصات والمتابعة وجد انه

The patient is a 40-year-old with triple-negative breast cancer (TNBC) undergoing neoadjuvant therapy (NACT). The initial plan was to give four cycles of taxane-carboplatin (TC) combined with pembrolizumab (Keytruda) followed by dose-dense anthracycline (AC) plus Keytruda, although the latter was not approved. The regimen was recorded as "C2 TXT CARBO".

Subsequent evaluation confirmed invasive ductal carcinoma, grade 3, triple-negative, Ki-67 80 %. Staging was cT2N1M0. BRCA testing identified a pathogenic BRCA1 mutation. The treatment plan was reiterated as TC plus pembrolizumab, followed by AC; dosing details included TC every 21 days, weight 86 kg, height 170 cm, with carboplatin AUC 5 pending Keytruda. Multidisciplinary discussion considered adding olaparib after surgery if no pathologic complete response, and prophylactic bilateral salpingo-oophorectomy.

On 12 April 2026 the patient received neoadjuvant Taxotere/Carboplatin (instead of the intended Taxol/Carboplatin). Pembrolizumab was refused by the Ministry of Health due to protocol violation. The plan was to resume the intended dose-dense AC for four cycles with G-CSF support, noting the pathogenic BRCA1 mutation.

A pharmacy note on 26 April 2026 documented the administration of adriamycin (ADRIA) 120 mg and endoxan (cyclophosphamide) 1200 mg.

On 17 May 2026 the patient did not receive Keytruda and continued dose-dense AC with G-CSF support for three cycles as directed by Dr Shaimaa; cycle 2 of AC was administered that day, with the next cycle scheduled for 31 May.

On 2 June 2026 the patient was scheduled for dose-dense AC cycle 3 (C3) with G-CSF and supportive care after laboratory review; however, the patient was deemed not fit for the cycle, which was postponed by one week with a plan to give filgrastim (Neupogen). The next visit was set for 9 June 2026.

Pharmacy documentation on the same day recorded an absolute neutrophil count of 0.8 ×10⁹/L, prompting the decision to postpone the cycle and administer filgrastim.

On 9 June 2026 the patient again received dose-dense AC cycle 3 with one dose of G-CSF and supportive care after labs; the previous cycle had been postponed.

On 30 June 2026 the record summarized the disease status: primary right breast cancer, clinical stage cT1N1M0, with a hepatic cyst; pathology confirmed invasive ductal carcinoma, grade 2, triple-negative, Ki-67 80 %, BRCA1 mutant. The patient had undergone an excisional biopsy on 7 September 2025 showing a 1.5 × 1 cm tumor with positive margins. Post-operative MRI showed bilateral BIRADS 3 fibroadenoma and right axillary

تحريرا في ,

التاريخ : 19-09-2026 **الوقت :**

الطبيب المعالج : هنا اسامه عبد الرحيم عبد البر

يعتمد ,

lymph node involvement. The treatment plan included 12 cycles of TC for clinical stage 4, followed by AC with adriamycin 120 mg and endoxan 1200 mg as neoadjuvant therapy.

After counseling on 2 August 2026, the patient expressed willingness to undergo risk-reducing bilateral oophorectomy and right completion mastectomy with sentinel lymph node biopsy, while deferring left mastectomy and reconstruction for financial reasons. Referral to the surgical clinic was arranged.

مركز النور المحمدي للأشعة
Alnour Almohammady Radiological Center
رنين مغناطيسي مفتوح - أشعة مقطعية - موجات صوتية - دوبلر ملون
22502599 - 22517341 - 26509782
Alnour Almohammady Mosque - Mosalla Square
Tel.: 26509782 - 22517341 - 22502599

Dear Prof. Dr :
Patient's name:

Date : 30 August 2025

BILATERAL SOFT TISSUE MAMMOGRAPHY AND BREAST U/S

- ACR c breast parenchymal density with nodulation impressive of adenosis
- No gross speculated masses or pathological calcifications on left side
- US revealed mottled echoing parenchyma with no solid lesions seen by US
- Right breast LIQ irregular ill defined lesion seen by US as irregular spiculated heterogeneous hypoechoic solid lesion measuring about 23 x 17 x20 mm with associated early ductal extension
- No other solid or cystic lesions seen by US on RT side
- LT sided likely nonspecific axillary LNS
- RT sided minimally suspicious axillary and UOQ LNS
- NO pathological supra clavicular LNS

CONCLUSION :-

- BIRADS IVc RT breast highly suspicious lesion as described for further management

MUCH OBLIGED YOURS.
Dr. OMNIA MOKHTAR M.D

Mammogram
[[DOCTYPE:MAMMOGRAM]]

GUSTAVE ROUSSY
INTERNATIONAL EGYPT

Patient Name: Rasha Mohamed Kamel Mohamed	Date : 11/11/2025
Exam : triphasic C.T Liver	Age : 41 Year(s)
Medical ID: 3112/2025	Pat. ID: 132859

MULTISCLICE C.T. EXAMINATION OF THE ABDOMEN AND PELVIS WITH TRIPHASIC STUDY OF THE LIVER AFTER ORAL AND I.V. CONTRAST INJECTION REVEALED :

Compared to the previous report dated 18/9/2025, the current study showed:

- No appreciable time interval changes regarding the size of the two previously noted left hepatic lobe non enhancing hypodense focal lesions largest still seen measuring 11 mm in diameter.
- Average sized liver showing homogenous parenchymal attenuation.
- No intra- or extra- hepatic biliary dilatation
- Patent and homogenously enhancing portal vein and its branches.
- Both kidneys are normal site, average sized and showing smooth regular cortical outline and preserved parenchymal thickness, nephrogram and excretory function. No stones or back pressure changes.
- Normal C.T appearance of the spleen, adrenal glands, pancreas, aorta and IVC.
- The urinary bladder shows no gross masses or diverticulae or stones.
- Normal appearance of the pelvic organs with no pelvic masses or collections.
- No abdominal or pelvic lymphadenopathy.
- No ascites.
- Midline anterior abdominal wall omentocele is noted.
- Clear lung bases.

Opinion:

- Stationary course regarding the previously seen Two left hepatic lobe focal lesions likely cysts.
- Anterior abdominal wall omentocele.

Dr. Sherif Wagdy

Dr. Sherif Wagdy

CT Scan
[[DOCTYPE:CT_SCAN]]

NILE
SCAN & LABS

مركز النيل للأشعة و التحاليل

Name : Rasha Mohammed Kamel Mohammed
Branch : Dokki
Ref.Physician Prof : Gostaf Roseh Hospital

Date : 05-11-2025 09:47
Medical No. : 241921
Ins.Company : تعاقد مستشفيات

$Tc^{99}m$ MDPSKELETAL SCINTIGRAPHY

History

- A 40-year-old patient, with history of breast cancer and referred for metastatic workup.

Technique

- Anterior and posterior whole body scans were acquired 2 hours post injection of the tracer.

Findings

- No abnormal active or cold focal bony lesions that could account for metastatic bone disease.
- Degenerative changes are noted at the spine and big joints.

Impression:

- The skeleton is free from metastases.

Best regards,
Dr. Alaa Nagi, MD
Nuclear Medicine

NILE DIAGNOSTIC CENTER

Call Center 19656

Bone Scan
[[DOCTYPE:BONE_SCAN]]

GUSTAVE ROUSSY
INTERNATIONAL

Patient Name	: الأستاذه / رشا محمد كامل	Request No.	: 13003905
Patient No.	: 28401200104102	Request Date	: 10/12/2025
Path No.	: 3829/25	Verification Date	: 16/12/2025
Sex	: Female	Financial Category	: جوستاف روسي - مبادره
Age	: 41 Y	Department	: Out Patient
Referreing Doc :		Registered by	: Mrs.Mayada Hegazy

Pathology Report

Clinical Data: Right breast scar site suspicious lesion ? recurrence.
Nature of Specimen: Tru-cut biopsy, collected on 10/12/2025, at 01:10 pm.

Gross Examination:
- Three fragmented grey white tissue cores, the longest is 1 cm. Totally submitted.

Technique:
- Sections prepared from paraffin block for immune-staining using Bench Mark ULTRA system for:
- ER clone (1E2), PR clone (SP1), Her2 clone (4B5) & Ki67 clone (30-9).

Microscopic Examination:
* Histological type: Recurrent invasive duct carcinoma.
* Grade 3 (differentiation 3, nuclear pleomorphism 3, mitosis 2).
* Ductal carcinoma in situ: Absent.
* Angioinvasion: Absent.
* Perineural invasion: Absent.

* IPT (Positive control showed appropriate reactivity):
- ER: Negative nuclear reaction in tumor cells (score 0/8).
- PR: Negative nuclear reaction in tumor cells (score 0/8).
- Her2: Negative membrane reaction in tumor cells (score 0).
- Ki 67%: 80%.

Diagnosis:
Right breast scar site lesion, Tru-cut biopsy:
- Recurrent invasive duct carcinoma, Grade 3.

IPT:
- ER: Negative (score 0/8).

Verified by :
Dr.Duaa Mohamed

CAP ACCREDITED
غرفة رقم (4) – الدور الأول – مبنى رقم (987) – كورنيش النيل – مصر القديمة – القاهرة – مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

Patient Name	: الأستاذه / رشا محمد كامل	Request No.	: 13003905
Patient No.	: 28401200104102	Request Date	: 10/12/2025
Path No.	: 3829/25	Verification Date	: 16/12/2025
Sex	: Female	Financial Category	: جو ـاف روسي - مبادره
Age	: 41 Y	Department	: Out Patient
Referreing Doc	:	Registered by	: Mrs.Mayada Hegazy

Pathology Report

- PR: Negative (score 0/8).
- Her2: Negative (score 0).
- Ki67%: 80%.

* Scoring system: "ER, PR"
ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66%
and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, week, 2, intermediate; and 3, strong). Total score from 0 to 8.
* Scoring system: "Her2"
Reference: ASCO-CAP Her2 Test Guideline Recommendations, 2013.IHC 0: as defined by no staining observed or membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within > 10% of the invasive tumor cells.
IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within >10% of
the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is complete, intense > 10% of tumor cells.

Signature
Dr. Walaa Ibrahem Ghoneim.
Dr. Heba El-Shafeey.

Verified by :
Dr.Duaa Mohamed

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Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

مركز النور المحمدى للأشعة
Alnour Almohammady Radiological Center
رنين مغناطيسى مفتوح - أشعة مقطعية - موجات صوتية - دوبلر ملون
مسجد النور المحمدى - ميدان المسلة ت : 22502599 - 22517341 - 26509782
Alnour Almohammady Mosque - Mesalla Square Tel.:26509782 - 22517341 - 225502599

Trans-Thoracic Echo

Patient Name: Rasha Mohamed
Date: 20-12-2025

M.MODE

LVEDD	48 mm	Left atrium	30 mm
LVESD	31 mm	Aortic root	27 mm
Ejection fraction	65 %	Posterior wall	09 mm
Fractional shortening	36 %	I.V septum	08 mm

Comment:

- Normal LV dimension and wall thickness with normal systolic function
- No RSWMA at rest
- Normal Aortic root and LA diameter
- Aortic Valve: Normal structure with Laminar flow, normal flow
- Mitral Valve: Normal structures with *trivial MR*
- Tricuspid Valve: Normal structures with *Trivial TR with RVSP =21 mmHg*
- Normal Right-side dimension with good systolic function (TAPSE=22 mm)
- Intact IAS and IVS
- Normal Pericardium
- No Intracardiac Mass

Dr. Waleed Atef, MD
Consultant Cardiology

Echo
[[DOCTYPE:ECHO]]

[[DOCTYPE:MDT_FORM]]
MDT Form

جوستافا روسي

مبادرة رئيس الجمهورية لدعم صحة المرأة المصرية

10/12/2026

MDT FORM

Name:	Rasha Mohamed Kamel Mohamed	**MRN:**	5067
Age:	43 Y	**ID:**	28401200104102
Menopausal Sta	pre	**Phone:**	01124480867
NRS SCORE:		**Marital Status:**	
BMI:		**Family History:**	
CO Morbidity:	free	**Reason Of MDT:**	post op

Clinical Data:

BX:

ER: **PR:** **HER2:** **KI67:**

Sonomammography:

Mets Workup:

History:

prem rt breast cancer ct1 n1 mo cts 2 hepatic cyst idc idc 2 triple -ve ki 80%
brca 1 mutant pt did excisional biopsy on 7/9/2025 mass 1.5x1 positive
margin mri breast postop bilat birads 3 fibroadenoma rt ax ln
pt rec nact 12 tc 4 ac last 30/6/2026

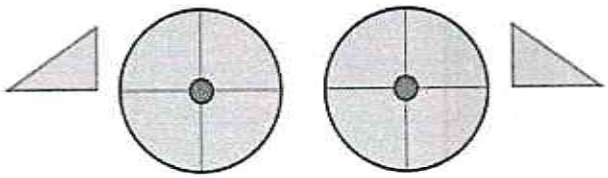
Recommendations:

pt underwent bilateral salpingo oophorectomy + completion mastectomy+
SLNB:
free ovaries breast : no residual invasive tumor

MDT Outcome & Plan

A case of TNBC BRCA1 mut, achieved PCR, for:
Follow-up (mammosonography alternating with MRI q 6 mths), with counselling on future contra-lateral risk reducing mastectomy.
P.S. In view of being initially cT1 N1, in PCR, and being BRCA1 mut...omit adjuvant Rth.

R L



TNM

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
د شيماء لاشين	د امنيه عيسوي				

12/10/2026 12/10/2026

GUSTAVE ROUSSY INTERNATIONAL EGYPTE

Gustave Roussy International-Egypt- Pathology Lab
جوستاف روسى الترناشونال – مصر معمل الباثولوجى

جوستاف روسي

Patient Name	رشا محمد كامل محمد	**Case No.**	**373085**
Sex/Age	**Female 42 Y**	**Patient No.**	**27750**
Registered Date	**07/9/2026 11:36:09 AM**	**Referred By Dr.**	**Surgery**
Validation Date	**04/10/2026 11:01:10 AM**	**Branch**	**Gustave Roussy**
Reporting Date	**11/10/2026 10:10:39 AM**	**Pathology No.**	**2879/26**

Pathology Report

Clinical Data: Right breast completion mastectomy (post-NACT) in a triple negative (BRCA positive) patient..

Nature of the specimen:
1- Right salpingo-oophorectomy., 2- left salpingo-oophorectomy., 3- Omental biopsy., 4- Right breast completion mastectomy., 5- Sentinel lymph nodes biopsy.

Gross Examination:
Five containers were received, designated as follow;
1- Right salpingo-oophorectomy: An ovary measured 4x3x1 cm, with attached tube 5 cm long.
2- Left salpingo-oophorectomy: An ovary measured 2.7x1.5x0.5 cm, with attached tube measured 6 cm long.
3- Omental tissue piece measured 6x6 cm, showed no nodules or indurations grossly.
4- Right completion mastectomy specimen measured 20x10x6 cm, with covering skin ellipse 15x10 cm. Nipple & areola looked unremarkable grossly.
* Serialling showed a tumor bed site measured 6.5x3.5x3.0 cm, located 3 cm away from the least deep margin.
5- Sentinel lymph nodes: Fibrofatty tissue piece measured 3.0x3.0 cm, entangling 3 lymph nodes, the largest measured 1.5x1x0.5 cm.

*** Block designation "20 Blocks":**
1:4- Right Tub & ovary., 5,6- Left ovary., 7,8- Left tube., 9:11- Omental biopsy., 12:17- Breast Tumor bed site., 18- Nipple & areola., 19- Deep margin., 20- SLNs.
- Gross was done by; Dr. Aya Ezzat.

**** Frozen (Intra-operative) results:**
Sentinel lymph nodes: Three free sentinel lymph nodes (0/3).

Microscopic examination:

GUSTAVE ROUSSY INTERNATIONAL Egypt

Gustave Roussy International-Egypt- Pathology Lab
جوستاف روسي انترناشونال – مصر معمل الباثولوجي

جوستاف روسي انترناشونال

Patient Name	رشا محمد كامل محمد	**Case No.**	**373085**
Sex/Age	**Female 42 Y**	**Patient No.**	**27750**
Registered Date	**07/9/2026 11:36:09 AM**	**Referred By Dr.**	**Surgery**
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1-Right salpingo-oophorectomy:
- Histopathological examination reveals ovary showing fibro-thecosis and retention cysts. The tube is widely congested with Para tubal cysts. The No malignancy.

2- Left salpingo-oophorectomy:
- Histopathological examination reveals ovarian tissue showing cystic follicle and retention cysts. The tube is congested. No malignancy.

3- Omental biopsy: Free omental fat with no malignancy.

4- Right completion mastectomy:

* **Histological type:** No residual viable remnants of the previously diagnosed invasive tumor with extensive therapy effect.
* **Tumor Cellularity:** 0%.
* **Ductal carcinoma in situ:** Absent.
* **Angioinvasion:** Absent.
* **Additional findings:** Extensive therapy effect.
* **Nipple & areola:** Free.
* **Deep margin:** Free.

5- Sentinel lymph nodes: Three free sentinel lymph nodes (0/3).

Diagnosis:

Right breast cancer (post-NACT):

1- Right salpingo-oophorectomy:
- Ovarian fibro-thecosis and retention cysts. Widely congested tube with Para tubal cysts. No malignancy.

2- Left salpingo-oophorectomy:
- Ovarian cystic follicle and retention cysts. Congested tube. No malignancy.

3- Omental biopsy:
- Free of malignancy.

4- Right completion mastectomy:
- No residual viable remnants of the previously diagnosed invasive tumor (Replaced by extensive therapy effect).
- Ductal carcinoma in situ: Absent.
- Nipple & areola: Free.
- Deep margin: Free.

[[DOCTYPE:SONOGRAPHY]]
Sonography

GUSTAVE ROUSSY INTERNATIONAL EGYPT

Gustave Roussy International-Egypt- Pathology Lab
جوستاف روسي انترناشونال – مصر معمل الباثولوجي

جوستاف روسي

Patient Name	رشا محمد كامل محمد	**Case No.**	**373085**
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5- Sentinel lymph nodes: Three free sentinel lymph nodes (0/3).
**** Histologic response to therapy: Pathological complete response to therapy.**
**** Stage: ypT0, ypN0.**

Signature
Dr. Walaa Ibrahim Ghonaim, MD.

CAP ACCREDITED